

ER Survival Guide

Things you should never do in your first Emergency Room shift

Smart Emergency Guide - educational quick reference. Always verify with local hospital protocol and senior clinical supervision.

Never...	Why it matters
Do not skip ABCDE	Stabilize airway, breathing, and circulation before chasing a final diagnosis.
Do not reassure chest pain too early	Exclude ACS, dissection, PE, pneumothorax, and esophageal rupture before benign labels.
Do not give nitroglycerin blindly	Check hypotension, RV infarct suspicion, severe aortic stenosis, and recent PDE-5 use.
Do not delay epinephrine in anaphylaxis	IM epinephrine is first-line. Antihistamines are not a substitute.
Do not ignore hypoglycemia	Altered mental status is glucose until proven otherwise. Check bedside glucose early.
Do not treat every wheeze as asthma	Heart failure, anaphylaxis, COPD, and pulmonary edema can mimic wheeze.
Do not miss hyperkalemia ECG danger	Peaked T waves or QRS widening with risk should trigger urgent senior help and therapy.
Do not intubate without a post-intubation plan	Prepare sedation, ventilation, confirmation, tube fixation, and hypotension management.
Do not give unlimited fluids to every shock	Reassess. Bleeding, cardiogenic, obstructive, and septic shock need different plans.
Do not discharge without safety-net advice	Give red flags, return precautions, medications, follow-up, and documentation.

ملاحظة عربية للفهم

الفكرة في أول مناوبة ليست أن تعرف كل شيء، بل أن لا تفوّت الخطر. ابدأ بترتيب التقييم الأولي، راقب العلامات الحيوية، اطلب المساعدة مبكراً، ولا تعطي دواء عالي الخطورة بدون تأكيد من الجرعة والطريق والبروتوكول المحلي.